Supreme Court of Ohio Clerk of Court - Filed March 14, 2024 - Case No. 2023-0936

**IN THE SUPREME COURT OF OHIO**

| | |
|---|---|
| STATE OF OHIO EX REL.<br>KATHRYN HUWIG,<br><br>Relator,<br><br>vs.<br><br>OHIO DEPARTMENT OF HEALTH, et al.<br><br>Respondents. | CASE NO.: 2023-0936<br><br>RELATOR'S RESPONSE TO RESPONDENTS' MOTION FOR JUDGMENT ON THE PLEADINGS |

Thomas W. Connors *
*Counsel of Record
Mendenhall Law Group
190 North Union Street, Suite 201
Akron, Ohio 44304
Tel: 330-888-1240
Fax: 330-762-9743
Email: tconnors@warnermendenhall.com

Attorneys for Relator Kathryn Huwig

Relator Kathryn Huwig submits the following response to respondents' motion for judgment on the pleadings.

## I. INTRODUCTION.

Respondents move for judgment on the pleadings under Civ. R. 12(C), explaining that such judgment is appropriate where construing a complaint as true, it is beyond doubt that relator can prove no set of facts entitling her to relief. Respondents' Motion for Judgment on the Pleadings ("Motion"), 14. However respondents then ignore the limitations of this rule, by attaching affidavits of its employees to their answer and arguing that they support judgment on the pleadings. Motion, 5-10, 17.

Respondents' answer to the complaint either admits, or fails to deny and therefore admits, essential elements of relator's claims for disclosure of public records. Civ. R. 8(D). Respondents admit that they are the public office and officials responsible for maintaining the requested public records, which include the Death and Vaccination Databases for 2021 (as defined in the Amended Complaint), as well as reports (the "Reports") from the Databases. Answer, ¶4. Respondents fail to deny relator's allegation that "the Death Database can be exported from the [Ohio Public] Warehouse as a .csv file." Amended Complaint, ¶20, Exhibit F, Priddle Affidavit, ¶10a ("the [secure] mortality dataset could be exported as a spreadsheet from the Warehouse…as a .csv file format…"). Respondents also admit that the Death Database can be downloaded in Excel, PDF and Word. Answer, ¶20. Respondents fail to deny the allegation that the Vaccination Database can be downloaded as a .csv file from the Data Ohio Portal. Amended Complaint, ¶38; Answer, ¶¶38-39. Respondents admit that they can redact these .csv files. Answer, ¶¶ 24, 38. Finally, respondents admit that relator told them they could "redact

what you consider to be exempted" (subject to relator's right to challenge improper redactions). Amended Complaint, ¶¶10, 12; Answer, ¶¶10, 12.

In their motion, respondents do not dispute that they have a duty under R.C. 149.43(B)(1) to provide all nonexempt information in requested public records, with exempt information redacted, as argued by relator in her Memorandum in Support of Amended Complaint for Writ of Mandamus ("Memorandum in Support"), 14-15. Further, respondents do not dispute that the Databases (as opposed to the requested Reports from the Databases) are public records as pointed out in relator's Memorandum in Support, 10-11. Instead, respondents only raise the issues of 1) whether the requested Reports from the Databases are public records and 2) whether the requests for the Reports and the Databases are overbroad.

As a result, respondents have conceded that the Databases are public records, and that the requested records can be provided with exempted information redacted. Relator will therefore focus on the remaining issues of whether the Reports are public records and whether the requests for the Reports and Databases are overbroad.

## II. The Requested Reports are Public Records.

Relator has already presented in her Memorandum in Support, the precedents showing that if a public office can "through reasonable programming produce the requested output, the record is already deemed to exist for purposes of R.C. 149.43." *Welsh-Huggins v. Jefferson County Prosecutor's Office*, 163 Ohio St. 3d 337, 356, 2020-Ohio-5371, ¶74. The issue raised by respondents is essentially whether searching a database is creating a new record for purposes of the Public Records Act. The answer throughout the country to this question is no:

> [C]ourts around the country appear to agree that public bodies are required to search and query their electronic databases for responsive records...Just as the public body is required to conduct a thorough search for printed records responsive to a records request, it is also obligated to search for already-existing records stored in a database. This search, even where it requires the creation of a new program or search mechanism, does

not constitute the creation of a new record but rather the public body's fulfillment of its duty to search and account for all responsive records. Kreienkamp, *The Meaning of Creation: Electronic Databases and Creating a Record to Fulfill a Public Records Request*, 47 Rutgers Computer & Tech. L.J. 197, 217-18 (2020).

It is evident that the information at issue has already been compiled into existing records by existing programming, given the data dictionaries for the Databases provided by respondents, and attached as Exhibits A and B to the Amended Complaint. Respondents have clearly admitted that "[s]taff had previously programmed each dataset (mortality…etc.) to report a particular set of variables in fixed positions to users with access to the secure dataset." Amended Complaint, Exhibit F, Priddle Affidavit, ¶10a. The data dictionaries attached as Exhibits A and B to the Amended Complaint reflect this previous programming. It therefore should be clear that querying these Databases is the functional equivalent of searching printed records in a file system i.e. respondent's duty to respond to a public records request.

Respondents argue that their computer systems "are not currently programmed to create" the reports requested by relator and are therefore not existing records. Motion, 19. Respondents explain that they "would be required to use third-party software and to export data from [their] database to a separate file." Motion, 19. Respondents claim that to provide the requested Death Database report they "would be required to use additional licensed software such as "'SAS' (SAS Institute Inc. Statistical Analysis System (SAS) software)" or "'SSMS'" (Microsoft SQL Server Management Studio)" which "are licensed for use by ODH." Answer, Exhibit 1, ¶3. F. For the Vaccination Database report, they claim they would be required to use specialized software such as "'SQL' (Oracle SQL Developer software)", and then "transfer this data to another format, such as a .csv file." Answer, Exhibit 3, ¶3. E.

Respondents are unduly mystifying the downloading of data reports from databases. A database stores data in multiple tables, each of which can contain multiple fields. Exhibit A,

Affidavit of Robert Bounds, ¶4. The database can be programmed to provide reports in .csv and other file formats. *Id.*, ¶12. As discussed above, respondents have admitted that the Databases are already programmed to provide reports which can be transferred as .csv files. Because of this existing programming, respondents are not required to use SAS, SSMS or SQL, described above, to obtain the requested records (although they certainly could use those applications). *Id.*, ¶20.

A database can also be opened by many different database management programs, including SAS, SSMS and SQL, and requested output could also be downloaded as .csv files, in the same format that output can be directly downloaded from the Databases. *Id.*, ¶20. CSV is a simple text file format, widely supported by a variety of database software programs, including common spreadsheet applications such as Microsoft Excel. *Id.*, ¶10.

The process of preparing a report involves writing a query and using search terms to designate the tables containing the desired fields. *Id.*, ¶9. The process is straightforward and uncomplicated and can be done in the present case in a short period of time, usually a matter of minutes. *Id.*, ¶¶11, 12.

To illustrate, the tables are the functional equivalent of file cabinets, and the fields are folders located in the cabinets. *Id.*, ¶8. The search query is the equivalent of notes showing a file clerk where to look for the requested records. *Id.*, ¶8.

The data dictionaries in this case are a list of the fields which make up the datasets described as the Death and Vaccination Databases. *Id.*, ¶12. These fields can be located in one or more tables in the Databases. *Id.*, ¶4. Relator has requested certain fields. Respondents can easily query the tables in the Databases for the requested fields using existing programming or their other software and provide the results in a report in a .csv file format. *Id.*, ¶13.

The query can be run on a copy of the online production system, without any impact on other users. *Id.*, ¶15. Running the query on the production system can also be done without impacting other users. *Id.*, ¶16. If there is any concern with directly querying the production system, the query can be scheduled to run after hours or late at night to avoid impacting other users. *Id.*, ¶16. Either approach can be used without impacting other users.

Given that the requested output can be produced through "reasonable programming," "record[s] are already deemed to exist for purposes of R.C. 149.43." Welsh-Huggins, ¶74.

Respondents' argument that relator is requesting "custom reports" is an obfuscation of the issues. Relator is requesting a report which can manifestly be produced through existing programming as respondents have admitted. Respondents' use of the term "custom reports" would describe all output that could be obtained from the Databases through use of a query.

The term "custom reports" would only be meaningful as a justification to deny a public record request when output is requested that can't be provided by existing programming. Respondents are suggesting that any query of a database would involve new programming. Such an interpretation would lock out all databases from public record requests.

Respondents also argue that requested reports and Databases include personally identifiable health information ("PHI"). PHI is exempted from disclosure under the Public Records Act. Respondents have the burden of proving that this exemption is squarely applicable, and this issue will be discussed below. Relator told respondents to redact what they considered to be exempt, subject to her right to challenge the redactions if not valid. In this circumstance, respondents have a clear duty to provide nonexempt information with redactions plainly visible. Respondents have failed to fulfill this clear duty.

The redaction process is also logically uncomplicated. The .csv files containing the requested reports and Databases can be opened in respondents Excel software, which can redact entire rows, columns and individual cells. *Id.,* ¶17. Respondents argue that the Vaccination Database file "would likely be too large to be transferred to a Microsoft Excel document. Answer, Exhibit 3, Napier Affidavit, ¶E.ii. Not so, Excel has a feature called Datamodel, which would permit the handling of the 16 million rows of data described by respondents. *Id.,* ¶18. Excel can easily redact by hiding exempt columns (fields) or rows (records), or respondents can use their database management software to achieve similar results. *Id.,* ¶17.

Searching, retrieving and redacting data are basic data management functions which fall well within respondents' duty to search for and provide public records.

## III. The Exemption for Protected Health Information ("PHI").

### A. The Disclosure Limitation Standard (Small Numbers Policy).

#### 1. The Issue of Individual Identification.

To apply an exemption from the Public Records Act, respondents must "plead and prove facts establishing ...that the requested records fall squarely within the exemption." *Welsh-Huggins*, ¶27. "Conclusory statements in an affidavit that are not supported by evidence are not sufficient evidence to establish the exemption's applicability." *Id.*, ¶30.

Respondents claim that the Death Database could be used to identify individuals and therefore discloses PHI. Motion, ¶5. Relator has not requested name and address information, but respondents claim that relator has requested certain fields in the Death Database including but not limited to zip code, race and date of birth that could be used to identify an individual. Amended Complaint, ¶30. They also claim that certain fields in the Vaccination Database, including date of birth and date of death could be used to identify individuals. However,

respondents have not provided any specific evidence proving that information in these fields would identify individuals.

Respondents argue generally that "identities of individuals may be discovered through 'triangulation' when small numbers within specific geographic areas and other demographic information are combined." Motion, p. 5. They also cite the ODH Disclosure Limitation standard known as the 'Small Numbers Policy.' However, they don't show how the fields at issue would lead to identification of individuals. Moreover, relator has produced evidence showing that the ODH Disclosure Limitation Standard did not preclude respondents from disclosing the fields at issue in 2019 and 2020 in the Ludlow and Miller cases, as well as to Robert Bounds. Exhibit A, ¶¶25-26.

**2. The Difference Between the Death Database (the Secure Mortality File) and the Public Portal.**

Respondents argue that they have made available a dataset from the Public Portal. They acknowledge the significant difference between the Death Database (the secure mortality file described in Exhibit A to the Amended Complaint) and the Public Portal dataset:

> While Ms. Huwig seeks reports with line-item entries for each decedent or each recipient of a COVID-19 vaccination as opposed to aggregate information, Ms. Huwig could nevertheless obtain useful information from the aggregate data. Review of the aggregate data could show if there is any correlation between the number of COVID-19 vaccinations in a particular county or in different demographic categories, and the number of deaths due to COVID-19, or other conditions, in those same counties or categories. Motion, 17-18.

Respondents further acknowledge the significant effect of the difference between the Death Database, with its line-item entries and the Public Portal dataset which only provides aggregate data. "[I]nformation is the meaningful organization of data. Individual data fields are meaningless. Data only becomes information when two or more data fields (or data points) are combined or 'related.'" Amended Complaint, Exhibit 2, ¶P.

Respondents openly acknowledge that they are withholding meaningful information by failing to provide the Death Database and attempting to substitute the aggregate information provided through the Public Portal. Relator gives a detailed explanation of how substituting aggregate information for line-item information hides meaningful information from the public. Exhibit B, Affidavit of Kathryn Huwig.

Furthermore, the Death Database and the Public Portal dataset are two different public records. As discussed above, a public record is deemed to exist when reasonable programming can produce the requested output. Providing one public record does not justify withholding another public record which they are also obligated to provide.

**3. The Institutional Review Board ("IRB") Process.**

Respondents also argue that they "referred Ms. Huwig to the Data Governance Committee and the Institutional Review Board if she wished to explore the research dataset process." Motion, 12. However, Catherin M. Stein, Ph.D., Professor of Epidemiology at Case Western Reserve University applied for approval of a research protocol on the Death and Vaccine Databases in March 2023 without any response. Exhibit B, Stein Affidavit, ¶¶5-6. The IRB acknowledged receipt, but despite reminders in June and October 2023, she has been ignored.

The IRB process does not supplant respondents' duties to respond to public records requests established by the Ohio Legislature in the Public Records Act. Respondents have a duty to provide nonexempt records with clearly marked redactions. Administrative regulations such as the IRB process can't override statutory requirements such as the Public Records Act. In any event the IRB process is meaningless as shown by Professor Stein's experience.

### B. Respondents' Failure to Redact and Provide Nonexempt Information.

Respondents have failed to prove the applicability of the PHI exemption and therefore disclosure is required under this Court's case law. However, even if they did present a colorable case for an exemption, they are premature because they have failed to provide the lawfully required nonexempt records with clearly marked redactions. Respondents have raised the potential of numerous redactions, which they may or may not make depending on their analysis under the Small Number Policy. Further, they are obligated to discuss such redactions with relator to see if she can cure any issues by modifying her request. This consultation process is not only required by law, it is a reasonable way to avoid unnecessary issues. Unfortunately, this Court will need to enforce this obligation given respondents' approach to this case.

## IV. Overbreadth.

Respondents cite the requirement of *State ex rel. Zidonis v. Columbus State Cmty. College*, 133 Ohio St. 3d 122, 2012-Ohio-4228, ¶26 that whether a request is overbroad "must be analyzed under the totality of facts and circumstances." Motion, 15. This requirement to do a deep analysis of factual issues would preclude respondents' request for judgment on the pleadings.

To support the overbreadth rule, respondents cite Zidonis' quote from *State ex rel. Warren Newspapers, Inc. v. Hutson*, 70 Ohio St. 3d 619, 624 (1994), which was based on *State ex rel. Zauderer v. Joseph*, 62 Ohio App. 3d 752, 756 (1989) (A request for "all traffic reports" was improper because it was "first unreasonable in scope and second, if granted would interfere with the sanctity of the record keeping process itself.")

Assessment of overbreadth must be tethered in some way to the wording of the Public Records Act. Considerations of clearly identifying the records sought, security of the records and interference with public operations are grounded in statutory language. However, "the mere

fact that a request may encompass a large number of records or may require a lengthy period for the public-records custodian to search and review the records before permitting the requester to access them does not render the request defective. See *Warren Newspapers*, 70 Ohio St. 3d at 624 (request to police chief for, inter alia, records of all internal investigations from 1988 to 1993 and all incident reports or traffic tickets written in 1992 not overbroad)." *State ex rel. Dehler v. Spatny*, 127 Ohio St. 312, 2010-Ohio-6711, ¶14 (J. Brown, dissent). As this Court recently held. If "the General Assembly wished to curtail public access to...records in...a broad categorical fashion, it would have included express language to that effect in the Public Records Act." *State ex rel. Summers v. Fox*, 163 Ohio St 3d 217, 2020-Ohio-5585, ¶35.

Moreover, it bears emphasis that *Summers* treated overbreadth as a public records exception, which the respondents have the burden of clearly establishing. *Id.*, ¶74. Relator is not seeking name and address information. Respondents have not presented sufficient evidence to prove the existence of the problems which the overbreadth rule addresses i.e. identifying records, protecting their security or preventing interference with operations.

Further respondents fail to recognize the difference between a compilation of information and the voluminous records from which that compilation is derived. The records sought by relator are databases and reports from databases. A database is a compilation of information, which has been held to be a public record separate and distinct from any underlying records. *Kish v. Akron*, 109 Ohio St. 3d 162, 169, 2006-Ohio-1244, ¶24. This Court has made clear that such databases are public records which must be disclosed by public offices without any hint of overbreadth concerns. *State ex re. Gambill v. Opperman*, 135 Ohio St. 3d 298, 2013-Ohio-761, ¶¶17, 26 (2013) (Requiring disclosure of county tax map database for period from January 2010 to September 2011).

Respondents argue that concerns regarding individual identification should be a factor in determining whether the request for the Death Database is overbroad. Motion, 17. However, this argument is belied by the fact that respondents have historically provided this database to the public without any concern for overbreadth. Exhibit A, Bounds Affidavit, ¶¶25-26; see also Ludlow and Miller cases.

Respondents also argue that the amount of data should be a factor in determining overbreadth. Motion, 16-17. However, the Databases and Reports are compilations, not the underlying records, and can be downloaded in a short period of time in an easily manageable format. Respondents are using the complexity of computers and database management software to obscure the simple reality that for purposes of this case they perform the basic functions of paper file systems. The overbreadth exception should not be used in a way that puts databases off bounds to public records requests.

The cases cited by respondents do not require such a result. *Zidonis* involved a fired employee seeking emails for a six-year period, where the employer did not have a system to retrieve by sender and recipient status. *Zidonis* did not revise her request to a shorter period as in the present case. These circumstances raised issues of identifying meaningful records and lack of existing programming that could do so. In the present case, there is no issue of identifying meaningful records or existing programming to do so.

*Dehler* involved a request for seven years of clothing and shoes receipts. Dehler refused to narrow the request or prepay the cost of copying, and prison officials showed that the request would "unreasonably interfere with the discharge of their duties." *Dehlers*, ¶313-14.

*State ex rel.Glasgow v. Jones,* 119 Ohio St. 3d 391, 2008-Ohio-4788 involved circumstances where a request for all emails and correspondence by date did not match the

public official's storage method. *Id.*, ¶16. *Glasgow* did require responses to requests identified by subject matter. *Id.*, ¶24.

In the present case, relator has limited her request to one year, and has identified the requested subject matter which is retrievable by reasonable programming. Moreover, providing the requested records would not unreasonably interfere with operations. Respondents have not met their burden of showing that relator's request falls squarely within the overbreadth exception.

## V. Attorney Fees, Statutory Damages and Costs.

Respondents can't reasonably dispute that the requested Databases are public records, given *Gambill's* holding that databases are public records. Nor can they reasonably dispute that the requested reports can be output by use of existing programming and therefore are also public records, given *State ex rel. Scanlon v. Deters*, 45 Ohio St. 3d 376, 379 (1986) and *Welsh-Huggins*, ¶74.

Further, respondents' arguments regarding PHI do not change their clear duty to provide nonexempt records with clearly marked redactions.

Finally, respondents' overbreadth argument fails to meet their burden to show that they fall squarely within this exception. There is no real issue regarding identifying the requested records or accessing them with existing programming. Relator revised her request to a reasonable time period and respondents thwarted any further revisions by failing to discuss such issues with relator. The issues respondents raise regarding efforts to output the records as .csv files and redact exempt information fall well within their clear duty to search for and provide public records.

Respondents' argument that amending relator's complaint was not an effective or efficient way to resolve the parties' dispute is not well taken. Motion, 23. Relator amended the

complaint mainly to add respondents' material admissions in the Standifer case. The allegations regarding the VAERS issues were to explain why the Vaccination Database had a death field. However, respondents' refusal to provide nonexempt information with redactions and to consult regarding revisions to relator's requests as required by statute, clearly constitutes a failure to employ an effective and efficient way to resolve the parties' dispute.

Accordingly, attorney fees, statutory damages and costs are warranted in this case.

Respectfully submitted,

*/s/ Thomas W. Connors*
Thomas W. Connors (0007226) *
*Counsel of Record
Mendenhall Law Group
190 North Union Street, Ste. 201
Akron, Ohio 44304
Tel: 330-888-1240
Fax: 330-762-9743
Email:tconnors@warnermendenhall.com

Attorneys for Relator Kathryn Huwig

**CERTIFICATE OF SERVICE**

I hereby certify that the foregoing has been served this 14th day of March 2024 via electronic mail, pursuant to Sup. Ct. Proc. R. 3.11, upon:

Katherine Bockbrader
Assistant Section Chief-Health and Human Services
Theresa R. Dirisamer
Senior Assistant Attorney General – Health and Human Services
Office of Ohio Attorney General Dave Yost
30 East Broad Street, 26th Floor

Columbus, Ohio 43215
Office number: 614-644-1734
Fax number: 877-690-1813
Katherine.Bockbrader@OhioAGO.gov
Theresa.Dirisamer@OhioAGO.gov

*/s/ Thomas W. Connors*
Thomas W. Connors (0007226)

**IN THE SUPREME COURT OF OHIO**

| | |
|---|---|
| STATE OF OHIO EX REL.<br>KATHRYN HUWIG,<br><br>Relator,<br><br>vs.<br><br>OHIO DEPARTMENT OF HEALTH, et al.,<br><br>Respondents. | CASE NO.: 2023-0936<br><br>AFFIDAVIT OF ROBERT BOUNDS |

Thomas W. Connors (0007226) *
*Counsel of Record
Mendenhall Law Group
190 North Union Street, Suite 201
Akron, Ohio 44304
Fax: 330-762-9743
Email: tconnors@warnermendenhall.com

Attorneys for Relator Kathryn Huwig

EXHIBIT
A

STATE OF TEXAS )
) SS
COUNTY OF MIDLAND )

Robert Bounds, after being duly sworn, states as follows:

**A. Background.**

1. I have worked as a data warehouse developer, as an independent contractor, for over 30 years. In that role, I have been responsible for designing, implementing, and maintaining data warehouses, which include relational databases.

2. I have extensive experience in the development of data warehouses in the health field. This experience includes data warehouse development projects for St. Joseph Hospital in Albuquerque, New Mexico for one- and one-half years, and for St. Vincent Regional Medical Center in Albuquerque for one- and one-half years.

3. I have reviewed relator's amended complaint and supporting memorandum, and respondents answer and motion for judgment on the pleadings in this case.

**B. The Basics of Relational Databases.**

4. Tables are the fundamental structure in a relational database. Each table represents a collection of related types of data. Columns, also referred to as fields, define the different types of data that can be stored in a table. An example of a column is found in the vertical lists set forth in Exhibit A to Relator's Amended Complaint.

5. Rows represent individual instances of data within a table. Each row, also referred to as records, contains a specific set of data elements corresponding to the fields (columns) defined for that table. An example of a row is found in the horizontal lists set forth in Exhibit A attached to Relator's Amended Complaint.

6. A data element is the content stored in a particular cell, which corresponds to a specific intersection of a row and column in a table.

7. Overall, rows (records) and fields (columns) in tables provide a structured way to organize and store data in a relational database, making it easier to manage and search (query) the data effectively.

8. To illustrate the process of searching relational databases, the tables are the functional equivalent of file cabinets, fields (columns) are the equivalent of folders in the cabinets, and rows are the documents in the folder. A search query is the equivalent of notes showing a file clerk where to look for the requested records.

**C. Preparing a Search Query.**

9. Preparing a query to search a relational database involves the use of table names, field names, key words and other criteria, which the database uses to access, retrieve and report data in a specified format such as a .csv file.

10. CSV is a basic text file format that uses commas to separate values (data elements). CSV is widely supported by Microsoft Excel and a variety of relational database software, including the SAS, SSMS and SQL programs described by respondents in their motion for judgment on the pleadings.

11. Preparing a query is a straight-forward process, which can be easily accomplished by a person with basic knowledge of relational databases and/or the database management application being used to access the database.

12. Respondents have acknowledged that they have programmed the Death and Vaccination Databases described in the pleadings in this case to provide reports that can be downloaded as .csv files. The data dictionaries provided by respondents, attached as Exhibits A

and B to the Amended Complaint describe the fields in their datasets. Respondents can prepare a query for the entire datasets for 2021, or reports containing relator's requested fields within a short time frame.

13. The operator then runs the query, and the computer will take one to fifteen minutes, depending on the computer's capacity, to process and provide the requested datasets or reports in a .csv file.

14. The type of computers necessary to hold the Vaccination Database would have ample memory and storage and processing capabilities to easily handle an ad-hoc query being run against the tables in the Vaccination Database.

15. A data warehouse is by definition an off-line system, which means that it is a copy of the production system. Any queries run on a data warehouse would have zero impact on the production system. Even if the data is not stored in a data warehouse, a copy of the production system can be made and a query run against it with zero impact on the production system.

16. Querying the production system itself would only take a few minutes and would not impact other users of the system. The database administrator, if concerned that querying the system may affect system performance, can schedule the query to run after hours or late at night to avoid any significant impact on users.

17. The resulting .csv files can then be opened by respondents' Excel software and any exempt or unrequested data can be redacted. Excel's can easily redact or hide fields (columns), rows or even individual cells.

18. The size of the Vaccination Database for 2021, which respondents describe as including 16 million rows of data, will not be a barrier to using Excel to redact the Database. Excel has a feature called Datamodel, which would permit it to handle 16 million rows of data.

19. The type of computers used to handle the Vaccination Database would have ample memory and storage and processing capabilities to easily handle an ad-hoc query being run against the tables in the Vaccination Database.

20. The SAS, SSMS and SQI database management applications described by respondents could also be used to access the Databases, although they are not necessary because they have already programmed their Databases to provide for .csv file output as discussed above.

21. Search queries through the described applications can also be prepared by people with basic relational database management experience. I am familiar with all the described applications and many more similar applications and could prepare such queries in a short time frame.

**D. The Disclosure Limitation Standard evaluation.**

22. Respondents state that their Disclosure Limitation Standard has been in effect for 20 years. Answer, Exhibit 2, ¶O.

23. The further state that "only when data becomes information can it be evaluated for confidentiality and other concerns." Answer, Exhibit 2, ¶P.

24. I received a copy of the Death Database from respondents for 2019 in connection with a project that I am working on.

25. The 2019 Death Database provided by respondents to me contained fields requested by relator in this case for the 2021 Death Database, which respondents are now claiming would permit individual identification.

26. Despite respondents' evaluation of the 2019 Death Database under the Disclosure Limitation Standard for confidentiality and other concerns, they did not determine that there existed any such concerns.

FURTHER AFFIANT SAYETH NAUGHT.

Robert Bounds

SWORN TO BEFORE ME and subscribed in my presence this 13TH day of March 2024.

KATY KUYKENDALL
My Notary ID # 128737009
Expires July 26, 2027

NOTARY PUBLIC

**CERTIFICATE OF SERVICE**

I hereby certify that the foregoing has been served this ___ day of _____, 2024 via electronic mail, pursuant to Sup. Ct. Proc. R. 3.11, upon:

Katherine Bockbrader
Assistant Section Chief-Health and Human Services
Theresa R. Dirisamer
Senior Assistant Attorney General – Health and Human Services
Office of Ohio Attorney General Dave Yost
30 East Broad Street, 26th Floor
Columbus, Ohio 43215
Office number: 614-644-1734
Fax number: 877-690-1813
Katherine.Bockbrader@OhioAGO.gov
Theresa.Dirisamer@OhioAGO.gov

*/s/ Thomas W. Connors*
Thomas W. Connors (0007226)

**IN THE SUPREME COURT OF OHIO**

| | |
|---|---|
| STATE OF OHIO EX REL. KATHRYN HUWIG,<br><br>Relator,<br><br>vs.<br><br>OHIO DEPARTMENT OF HEALTH, et al.,<br><br>Respondents. | CASE NO.: 2023-0936<br><br>AFFIDAVIT OF KATHRYN HUWIG IN SUPPORT OF RESPONSE TO RESPONDENTS' MOTION FOR JUDGMENT ON THE PLEADINGS |

Thomas W. Connors (0007226) *
*Counsel of Record
Mendenhall Law Group
190 North Union Street, Suite 201
Akron, Ohio 44304
Fax: 330-762-9743
Email: tconnors@warnermendenhall.com

Attorneys for Relator Kathryn Huwig

EXHIBIT
B

STATE OF OHIO )
) SS
COUNTY OF CUYAHOGA )

Kathryn Huwig, after being duly sworn, states as follows:

1. ODH's response clarifying the difference between data and information is that the relationships between data points is key in understanding the importance of the outcome of this suit. The Ohio Department of Health has access to all of the data collected in regards to the COVID-19 pandemic, including all of the relationships between data points. They are also refusing to release line-item data, even to established researchers in this state. Instead, they offer aggregate data, with nearly all of the important information stripped from it. And when too much information was gleaned from what they did release, as described in my testimony for HB90 in February 2021, the response was not to offer explanation or clean up the issues, but rather to break the relational bonds between data points so as to prevent me or any other observers from identifying that most reported "COVID" deaths made no logical sense as being from an acute respiratory illness.

2. The disassociation of the mortality data from the hospitalization and infection data is not the only place that ODH used its monopoly on the COVID-19 data to further aims that are not aligned with the well-being of Ohioans. A specific, high-profile example of the consequences of only aggregate data being available to the public can be found within the Vax-a-Million initiative announced by Gov. DeWine on May 12th, two days after 12–15-year-old children were allowed to receive the shots for the first time.

3. To be clear, Vax-a-Million is only one specific example of how a controlled and partial release of the public data has affected the response to COVID-19 in Ohio and nationwide.

Similar obfuscations of the publicly presented data occurred in all other aspects of the pandemic too, including the number of deaths truly attributable to COVID-19, the number of 'cases' and hospitalizations throughout the state that led to many wide-ranging and often devastating consequences for the citizens of Ohio throughout the pandemic.

4. The Ohio Department of Health made COVID-19 related data public on the coronavirus.ohio.gov website, including information on cases, hospitalizations, deaths, and vaccinations. A small amount of this information was downloadable using a csv file, including the file I had used to do the analyses on the alleged "COVID"-related mortality for my HB90 testimony. However, a lot more information was available only on the website itself, either graphically or by mousing over different elements of the interactive graphs. This extra data included race, age, ZIP code, 'probable' case designations and more.

5. The displayed (but not downloadable) data was updated and overwritten every day, so if an individual visited the site on any given day they would only be able to see a cumulative number, not how that number changed over time. As an illustration, attached is a screenshot of the vaccination website interface on coronavirus.ohio.gov Exhibit B-1. This screenshot was taken of the data posted on 3/23/23. Of particular interest here is the section showing the breakdown of vaccines started by age, which shows the *cumulative* number of vaccines administered to date to that age group. This is a single data point and of limited use.

6. However, due to the diligence of dozens of Ohioans who volunteered many tens of thousands of hours to collect undownloadable data by hand for every county, every day, for years, we are able to assemble some of the information that ODH has, but isn't releasing.

7. The process for the vaccine by age data collection was simply to record the total number of vaccines given every single day for each county and to track the daily changes over

time by subtracting the prior day's total from the current day's total. We could then graph the difference in totals for each age group to see exactly which age group was getting vaccinated and when. For instance, on 3/23/23 there were 1,004,684 0-19 year-olds, total, who had taken the shot. A copy of the graph published by ODH is attached as Exhibit B-1. One week prior, on 3/16/23, there were 1,003,920 0-19 year-olds who had taken their first dose. From this, we can subtract the two totals and infer that 764 0-19 year-olds received their first dose in the week between 3/16/23 and 3/23/23. This is an imperfect system because sometimes it took days, weeks, or even months for vaccinations to be reported to the state. But overall, it gave a reasonable approximation for what age groups were being vaccinated and roughly when.

8. By the time the Vax-a-Million was announced by Gov. DeWine on May 12, 2021, the group of dozens of volunteers had collected approximately 5 weeks of age-stratified data, allowing for a reasonable baseline up until that point. Shortly after his announcement of the Vax-a-Million, Gov. DeWine and his administration declared it a success. However, when looking at the graph created using the previously described method, it was clear that any increase in vaccine uptake was entirely, or almost entirely driven by individuals under the age of 20 — the vast majority of whom were ineligible to enter the contest for the million-dollar prizes. Attached is the post I made on May 20, 2021, 8 days after the Vax-a-Million announcement, clearly showing that the increase was almost entirely within the youngest age group. Exhibit B-2, attached.

9. Although we were limited in how we could approximate the data, ODH has direct access to the information. They have the exact date of administration as well as the birthdate (age) of every individual. They have a very clear and accurate answer as to who was getting vaccinated and when. With the way the data was presented on the website, it is unreasonable to

assume that anyone would be able to check the accuracy of their statements, except via the difficult and incredibly time-intensive process described above.

10. In fact, two sets of researchers have tried to study the situation, coming to conflicting conclusions. The first study was published in JAMA in August of 2021 by researchers from the Boston University School of Medicine. The researchers compared vaccine uptake in Ohio vs other states without a lottery and found that uptake is similar. From this, they inferred that the observed increase was likely due to 12-15 year olds having access to the vaccine at the same time - a situation that existed in all states. The second study was performed at the Institute for Applied Computational Science at Harvard University, who used modeling methods to calculate the theoretical difference between Ohio's actual vaccination numbers with that of a calculated 'synthetic Ohio'. The second paper neglected to address the impact of 12-15 year-olds and came to the conclusion that the Vax-a-Million increased new vaccinations by 114,553. To my knowledge, no studies exist on the actual impact of vaccine uptake by 12-15 year-olds on the increase in to vaccinations in Ohio, because it would be impossible to go back in time and collect the data I and my volunteers did. To my knowledge, ODH has never released that information to any other entity, despite the lack of any PHI associated with it.

11. Another activity ODH has consistently performed throughout the pandemic is to use confusing language and percentages to imply something is either positive or negative, instead of using plain language.

12. On 5/20/21, ODH posted a press release touting the 'success' of the Vax-a-Million program. This was the same day as I posted showing that the clear increase lies with

individuals under the age of 20 - most of whom were ineligible for the million dollar prizes. The press release provided in part:

**According to Ohio Department of Health data, vaccinations in those ages 16 and older:**

- **Dropped by 25% the weekend of May 7 to May 10, compared to the weekend of April 30 to May 3.**
- **Increased by 28% for the same age group from May 14 to May 17, the weekend following the announcement of the Vax-a-Million drawings.**

13. This language is not only confusing, but it is directly misleading. They have not offered any numbers, only percentages which is highly deceptive. For illustration of how this works, if, for ease of calculation, we use 100 vaccinations as our theoretical starting point on 4/30/21-5/3/21, when we drop by 25% we come up with 75 vaccinations on the weekend of 5/7/21-5/10/21. For the second line, we now start with 75 in our theoretical situation. When we add 28% of 75 (which is 21) we end up with 96 - which represents a 4% decrease from our starting point on 4/30/21-5/3/21. They also cherry-picked three "weekends" that encompass 4 days each. Why not include the other three days? Why did they not compare week over week changes, which are easier and clearer to understand? Why did they not include any actual numbers to help Ohioans understand the true scale of what was happening? This is not the behavior we should expect from the stewards of the most important information to be had during a global pandemic.

14. None of this data, nor any relational 'information' described here is PHI, and ODH has had full access to this data the entire time.

15. The purported 'success' of the Vax-a-Million initiative increased Gov. DeWine's profile internationally during a time when he was running for re-election as governor of Ohio. The Director of the Ohio Department of Heath is a direct political appointee of the governor. Critical information relating to the timing and age distribution of individuals taking the shot was

concealed or presented in a very confusing manner in press releases while Gov. DeWine made many public statements about its 'success' on the national stage, inducing other states and even other countries to adopt similar programs. To my knowledge, there has been no release of information from ODH to corroborate these claims.

16. In this affidavit I have focused exclusively on the Vax-a-Million initiative because it is a relatively simple and easily understood concealment of information with national implications as other states followed suit. It has clear political benefits to Gov. DeWine and therefore his political appointees, including Dr. Vanderhoff, party to this writ of mandamus, current Director of the Ohio Department of Health and who was the Chief Medical Officer of ODH at the time of the program.

FURTHER AFFIANT SAYETH NAUGHT.

Kathryn Huwig

SWORN TO BEFORE ME and subscribed in my presence this 13th day of March 2024.

NOTARY PUBLIC

NATHANIEL ENGLISH JR, NOTARY PUBLIC
COMM. EXP. NOV. 06, 2028
STATE OF OHIO

| Vaccine Started[1], Change from Last Week[4] | Vaccine Started[1] By Population | Vaccine Started[1], Total |
|---|---|---|
| 1,543 | 64.82% | 7,577,399 |



**Key Metrics**

Select to view key demographic groups

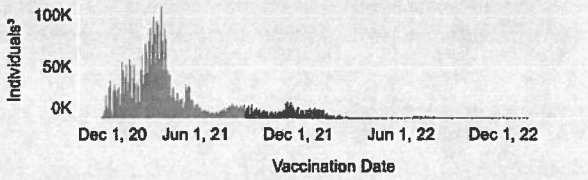
Age Group

| | | |
|---|---|---|
| 0-19 | 34.78% | 1,004,684 |
| 20-29 | 60.77% | 943,792 |
| 30-39 | 66.06% | 976,017 |
| 40-49 | 69.62% | 965,791 |
| 50-59 | 75.20% | 1,159,765 |
| 60-64 | 83.37% | 664,047 |
| 65-69 | 90.51% | 603,672 |
| 70-74 | 95.30% | 497,860 |
| 75-79 | 91.08% | 325,686 |
| 80+ | 87.31% | 436,085 |

**View Count By**

Select to view counts by daily or cumulative

Daily Total

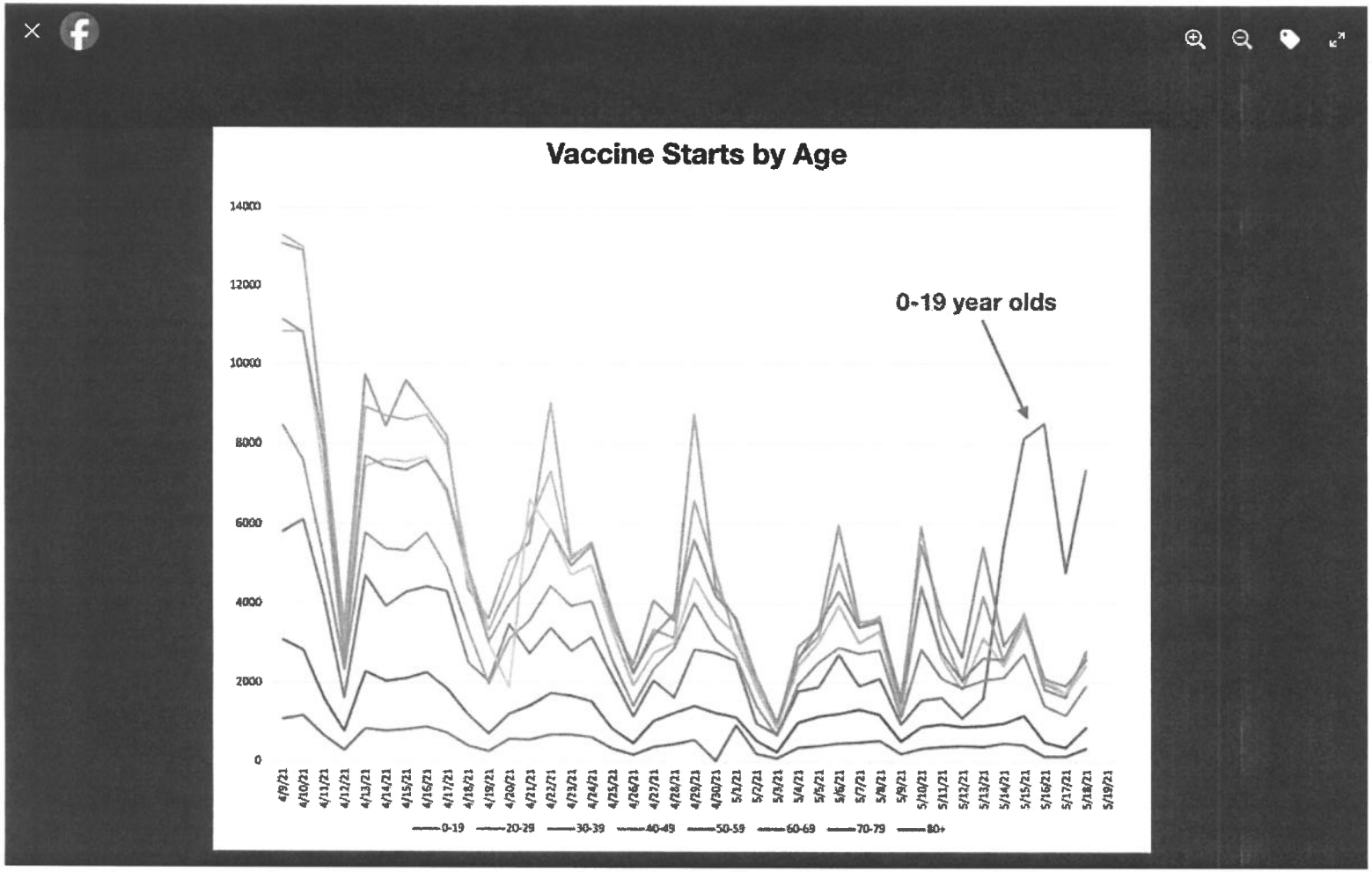



Residency Not Reported
8,656

Residents of Other States - Vaccinated in Ohio
307,032

EXHIBIT
B-1

EXHIBIT B-2




**Kathryn Huwig**
May 20, 2021 ·

## Vax-a-million

So the propaganda out there right now is thick that the Vax-a-million drive is successful and has been driving up the numbers.

And it would be impossible from the data available to us currently on http://coronavirus.ohio.gov to be able to determine the real truth of those statements. The csv file ***only*** gives us the raw totals per county. It does not give us the age data. But that age data is available on the website via the graphical interface. The graphical interface is dynamic, so every day the new information replaces the day before's. They give us the data without giving us the data, basically.

But happily for us, I have become an obsessive data hoarder, and I have a group of absolutely amazing, dedicated people who have been thanklessly and precisely culling that data from the graphical interface for the last 6 weeks. Because of all that work, we can look back in time now and see the ***real*** story.

Behold - attached, you may find the data on vaccine starts *reported* by age over the last 6 weeks. Again, these are *reported* dates, not assigned, and we already know that they have been adding in new starts and completions all the way back to December in the month of May. But the signal is unequivocal. The increase in new v-starts are being almost entirely driven by those under the age of 20.

Edit

333 308 78

Like Share

**IN THE SUPREME COURT OF OHIO**

| | |
|---|---|
| STATE OF OHIO EX REL. KATHRYN HUWIG,<br><br>Relator,<br><br>vs.<br><br>OHIO DEPARTMENT OF HEALTH, et al.,<br><br>Respondents. | CASE NO.: 2023-0936<br><br>AFFIDAVIT OF CATHERINE M. STEIN, PH. D |

Thomas W. Connors (0007226) *
*Counsel of Record
Mendenhall Law Group
190 North Union Street, Suite 201
Akron, Ohio 44304
Fax: 330-762-9743
Email: tconnors@warnermendenhall.com

Attorneys for Relator Kathryn Huwig

EXHIBIT
C

STATE OF OHIO )
) SS
COUNTY OF CUYAHOGA )

Catherine M. Stein, Ph. D after being duly sworn, states as follows:

**A. Background.**

1. I am a Professor of Epidemiology at Case Western Reserve University's Department of Population and Quantitative Health Sciences. A copy of my curriculum vitae is attached as Exhibit C-1.

2. I conduct an active research program and am currently principal investigator on a number of research protocols.

3. An institutional review board (IRB) is the institutional entity charged with providing ethical and regulatory oversight of research involving human subjects, typically at the site of the research study. I am IRB certified to conduct human subjects research.

4. I am aware of the Ohio Department of Health's Data Governance Council and its requirement that all research activities regarding its records be reviewed and approved by an Institutional Review Board.

5. I prepared a research protocol regarding the death and vaccination records which are the subject of Kathryn Huwig's public record request in this case. I submitted this protocol and other required documents to a designated IRB representative for approval on May 9, 2023. A copy of the protocol and email application are attached as Exhibit C-2

6. On May 17, 2023 I was informed by the IRB representative that my application was received and was being reviewed.

7. I followed up with the IRB representative on June 8, 2023 and October 23, 2023 regarding the status of my application for IRB approval, but did not receive a response. To date I

have not received any response from the IRB regarding my application other than confirmation that it was received.

FURTHER AFFIANT SAYETH NAUGHT.

Catherine M. Stein, Ph. D

SWORN TO BEFORE ME and subscribed in my presence this 13th day of March 2024.

XZIA B JOHNSON
Electronic Notary Public
Commonwealth of Virginia
Registration No. 7883496
My Commission Expires Apr 30, 2024

Xzia B. Johnson
NOTARY PUBLIC

This notarial act was performed: (check one) ☐ in person ☒ remote notarization

In

**Catherine M. Stein (Jedrey), Ph.D.**
Department of Population and Quantitative Health Sciences[1]
Case Western Reserve University
Wolstein Building Rm 1316
2103 Cornell Rd, Cleveland, OH 44106-7281
*phone:* (440) 752-7330, *email:* kasia6901@yahoo.com

2/20/23

## EDUCATION

**2006** Structural Equation Models and Latent Variables: An Introduction
Inter-university Consortium for Political and Social Research
Ann Arbor, MI

**2005-2007** Case Western Reserve University
Cleveland, OH
Clinical Research Scholars Training Program

**1999-2004** Case Western Reserve University
Cleveland, OH
Ph.D. in Genetic and Molecular Epidemiology
*Dissertation: Genetic and Environmental Influences on Tuberculosis Susceptibility*
*Advisor: Dr. Sudha K. Iyengar*

**2003** International Workshop on Methodology of Twin and Family Studies
University of Colorado
Boulder, CO

**2001** Advanced Genetic Linkage Analysis Course
Rockefeller University
New York, NY

**1995-1999** John Carroll University
University Heights, OH
B.S. in Biology, minors in statistics and mathematics, with honors

## ACADEMIC AND PROFESSIONAL EXPERIENCE

**2021 -** Professor
Department of Population and Quantitative Health Sciences
Case Western Reserve University
Cleveland, OH

**2012 - 2021** Associate Professor
Department of Epidemiology and Biostatistics,

[1] Department changed name to Population and Quantitative Health Sciences in May 2017

EXHIBIT
C-1

Case Western Reserve University
Cleveland, OH

**2006-2012** Assistant Professor, Division of Genetic Epidemiology
Department of Epidemiology and Biostatistics,
Case Western Reserve University
Cleveland, OH

**2005-2006** Senior Instructor, Division of Genetic Epidemiology
Center for the Modern Epidemiology of Infectious Diseases, and
Department of Epidemiology and Biostatistics,
Case Western Reserve University
Cleveland, OH

**2004-2005** Research Associate, Division of Genetic Epidemiology
Center for the Modern Epidemiology of Infectious Diseases
Department of Epidemiology and Biostatistics,
Case Western Reserve University
Cleveland, OH

**2003-2004** Research Assistant II
Department of Epidemiology and Biostatistics
Case Western Reserve University
Cleveland, OH

**2002-2004** Independent Contractor / Consultant
Division of Pulmonary and Critical Care
University Hospitals of Cleveland
Cleveland, OH

**2000-2003** Pre-doctoral Fellow, Division of Genetic and Molecular Epidemiology
Department of Epidemiology and Biostatistics
Case Western Reserve University
Cleveland, OH

**1997-2000** Research Assistant, Center for Adolescent Health
Department of Epidemiology and Biostatistics
Case Western Reserve University
Cleveland, OH

**1995-1999** Teaching assistant, tutor, and grader
Department of Mathematics and Computer Science
John Carroll University
University Hts., OH

## Secondary Appointments

| | |
|---|---|
| 2013 - | Center for Proteomics & Bioinformatics, Case Western Reserve University |
| 2017- | Division of Infectious Diseases, Department of Medicine, Case Western Reserve University |

## MEMBERSHIP IN PROFESSIONAL SOCIETIES

| | |
|---|---|
| **2011-present** | International Union Against Tuberculosis and Lung Disease |
| **2000-present** | American Society of Human Genetics |
| **2000-present** | American Statistical Association |
| **2000-present** | International Genetic Epidemiology Society |
| **2004-present** | African Society of Human Genetics |
| **1998-1999** | Mathematical Association of America |

## HONORS AND AWARDS

| | |
|---|---|
| **2020** | Flora Stone Mather Center for Women Research and Professional Development Grant<br>Case Western Reserve University |
| **2017** | Award of Tenure<br>Case Western Reserve University |
| **2016** | ADVANCE Opportunity Grant<br>ACES+ Initiative<br>Case Western Reserve University |
| **2011** | Editors' Award for the Language section of the *Journal of Speech, Language, and Hearing Research*, for "Literacy Outcomes of Children With Early Childhood Speech Sound Disorders: Impact of Endophenotypes" by Lewis et al. **(Stein senior author)** |
| **2011** | Nominee: Athena Young Professional Award<br>*Inside Business* Greater Cleveland / Akron area |
| **2009** | ADVANCE Opportunity Grant<br>ACES+ Initiative<br>Case Western Reserve University |
| **2005-2009** | Clinical Research Scholar<br>Case/Cleveland Clinic Clinical Research Training Program<br>(Roadmap K12/KL2) |
| **2001-2002** | Student of the Year Award<br>Case Western Reserve University<br>Division of Genetic and Molecular Epidemiology |

| | |
|---|---|
| **2002** | Nominee: Marcus Singer Award<br>Case Western Reserve University School of Medicine |
| **2001** | Honorable Mention, Research Days Oral Presentation Competition<br>MetroHealth Medical Center |
| **2000-2003** | Predoctoral Fellow<br>Case Western Reserve University<br>Division of Genetic and Molecular Epidemiology |
| **1999** | Honors Scholar<br>John Carroll University |
| **1999** | Phi Eta Sigma<br>John Carroll University |
| **1997-1999** | Pi Mu Epsilon<br>Alpha Epsilon Delta<br>John Carroll University |
| **1998** | Excellence in Slovak Language<br>John Carroll University |

**RECENT PEER REVIEWED PUBLICATIONS (out of 95 total peer-reviewed publications, as well as 14 reviews/book chapters and 3 commentaries/letters to editor)**
https://orcid.org/0000-0002-9763-5023

**Stein CM**, Benchek P, Igo Jr RP, Sobota RS, Chervenak K, von Reyn CF, Lahey P, Boom WH, Bush WS, Marsit C, Scott WK, Sirugo G, Williams SM (2021). Methylome-wide analysis reveals epigenetic marks associated with resistance to tuberculosis in HIV-infected individuals from East Africa. J Infect Dis, 5;224(4):695-704.

Campo M, Heater S, Peterson GJ, Simmons JD, Skerrett SJ, Mayanja-Kizza H, **Stein CM**, Boom WH, Hawn TR (2021). HDAC3 inhibitor RGFP966 controls bacterial growth and modulates macrophage signaling during Mycobacterium tuberculosis infection. Tuberculosis, Tuberculosis, 127:102062. doi: 10.1016/j.tube.2021.102062.

Thiel BA*[2], Worodria W, Nalukwago S, Nsereko M, Sanyu I, Rejani L, Zawedde J, Canaday DH, **Stein CM**, Chervenak KA, Malone LL, Kiyemba R, Silver RF, Johnson JL, Mayanja-Kizza H, Boom WH (2021). Immune cell distribution in bronchoalveolar lavage fluid of Ugandan adults who resist versus those who develop latent Mycobacterium tuberculosis infection. PLoS One 16(4): e0249477. https://doi.org/10.1371/journal.pone.0249477

*I helped design the study, reviewed the statistical analysis, and edited the paper.*

[2] * Indicates student as first author

Simmons JD, Van PT, **Stein CM**, Chihota V, Ntshiqa T, Peterson GJ, Benchek P, Velen K, Fielding K, Grant A, Graustein AD, Nguyen FK, Gottardo R, Mayanja-Kizza H, Wallis RS, Churchyard G, Boom WH, Hawn TR (2021). Monocyte metabolic programs are associated with resistance to TST/IGRA conversion. J Clin Invest 131(14):e140073

Benchek P, Igo Jr RP, Voss-Hoynes H, Wren Y, Truitt B, Freebairn L, Tag J, Chan ER, Lewis BA, **Stein CM**[3], Iyengar SK (2021). Association between loci regulating brain and neural pathways and quantitative traits of communication disorders. npj Genom. Med. 6, 64 https://doi.org/10.1038/s41525-021-00225-5

Lewis BA, Benchek P, Tag J, Miller G, Freebairn L, Taylor HG, Iyengar SK, **Stein CM**. Psychosocial Co-morbidities in Adolescents With Histories of Childhood Apraxia of Speech. American Journal of Speech-Language Pathology, Accepted 2021

McHenry ML*, Wampande EM, Joloba ML, Malone LL, Mayanja-Kizza H, Bush WS, Boom WH, Williams SM, **Stein CM** (2021). Interaction Between M. tuberculosis Lineage and human genetic variants reveals novel pathway associations with severity of TB. Pathogens, 10(11):1487. doi: 10.3390/pathogens10111487 PMCID: PMC8617877

McHenry ML*, Benchek P, Malone LL, Nsereko M, Mayanja-Kizza H, Boom WH, Williams SM, Hawn TR, **Stein CM** (2021). Resistance to M. tuberculosis infection: heritability and genome-wide association study. EBioMedicine 74: 103727. https://doi.org/10.1016/j.ebiom.2021.103727

Simmons JD, Dill-McFarland K, **Stein CM**, Van PT Chihota V, Ntshiqa T, Maenetje P, Peterson GJ, Benchek P, Nsereko M, Velen K, Fielding KL, Grant AD, Gottardo R, Mayanja-Kizza H, Wallis RS, Churchyard G, Boom WH, Hawn TR (2022). Monocyte Transcriptional Responses to Mycobacterium tuberculosis Associate with Resistance to Tuberculin Skin Test and Interferon Gamma Release Assay Conversion. mSphere 7(3):e0015922 PMCID: PMC9241521

Dawkins BA, Garman L, Cejda N, Pezant N, Rasmussen A, Rybicki BA, Levin AM, Benchek P, Seshadri C, Mayanja-Kizza H, Iannuzzi MC, **Stein CM**, Montgomery CG (2022). Identification of Novel HLA Alleles Protective Against TB and Persistent Sarcoidosis Using Nearest-neighbor Feature Selection. Genet Epidemiol 46(7):463-474.

**Pediatric TB Contact Studies Consortium**[2] (2022). Childhood BCG Vaccination and Risk of Pulmonary and Extrapulmonary Tuberculosis Throughout the Life Course: A systematic review and Individual-participant Meta-analysis of 24 Cohort Studies. Lancet Public Health 10: E1307-1316.

Novembre J[•], **Stein C**[•], Asgari S, Gonzaga-Jauregui C, Landstrom A, Lemke A, Li J, Mighton C, Taylor M, Tishkoff S (2022). ASHG Guidance: Addressing the challenges of polygenic scores in human genetic research. Am J Hum Genet- 109: 2095-2100

---

[3] Co-senior author

**From:** Cathy Stein <kasia6901@yahoo.com>
**To:** Michelle Niesley <docmniesley@gmail.com>; Steve Kirsch <stk@skirsch.com>
**Sent:** Tuesday, May 9, 2023 at 04:28:13 PM EDT
**Subject:** stuff for IRB

Hi Michelle,

Sorry for the extreme delay - I've been spending my time contacting different states trying to figure out what data is available, which has sort of affected the design and data request. Some states have said that IRB approval will be required. I'm attaching our draft protocol, my signed CV for the file, and human subjects certification for the file. Please take a look at the protocol and see if I need to add anything, reformat, clarify, etc. Otherwise, please let me know next steps.

cathy

**Catherine M. Stein, Ph.D.**
Professor of Epidemiology

LifeAndLibertyOH
@liberty_oh
Instructor, IPAK-edu, Basic Principles of Epidemiology

***If you are a real person and want to make sure I see your email, please send to cmstein@protonmail.com

EXHIBIT
C-2

## Trends in COVID-19 mortality and vaccination during the pandemic

**PI: Catherine Stein, Ph.D**

### Specific Aims

1. To examine mortality rates during the COVID-19 pandemic. We will examine both all-cause mortality by age group and cause-specific mortality between 2020-present, to see how causes of mortality have changed by age group.
2. To examine how mortality rate varies by vaccination in the individual. We will examine the number of COVID-19 vaccinations received by the individual to see how mortality rate, both all-cause and cause-specific as stated in Aim 1, differ.

To achieve these aims, we will be requesting the national death index data collected by the CDC (https://www.cdc.gov/nchs/ndi/index.htm) and the state by state immunization records (https://www.cdc.gov/vaccines/programs/iis/contacts-locate-records.html). We are requesting death data back to 2017 so that trends prior to the COVID-19 pandemic can be compared to trends after the pandemic. These two databases will be merged on whatever identifying information is available. States vary by which datafields are recorded with vaccination record, so we will utilize the most identifiable information available. Once the databases are merged, the linking information will be removed and destroyed so that personal health information is no longer available to the analysts, thus maintaining human subjects protections.

### Requested datafields – data availability varies by state

Social security number

Date of birth

Date of death

Cause of death

Age at death

Date of vaccination(s) – original series and follow-up boosters

Vaccine manufacturer for each vaccine administered

State of residence

County of residence

Phone number

Address

### Analysis Plan

### Aim 1

For this analysis, we will estimate the cause-specific mortality rates, by age group, for each year from 2018-2022. We will compare these mortality rates pre- and post- pandemic, to examine trends in causes of mortality.

**Aim 2**

For this analysis, we will calculate the overall mortality rate in the vaccinated vs. unvaccinated, stratified by age group and also by number of vaccines received. We will then repeat this analysis for cause-specific mortality rates.

**Background**

The COVID-19 pandemic has been a major cause of mortality globally since 2020. Not only have there been a significant number of COVID-19 associated deaths, particularly among the elderly, immunosuppressed, and individuals with pre-existing conditions, there have also been a surge in other causes of death [1]. In that early cut of the mortality data, the CDC showed that there was an increase in deaths due to cardiovascular and Alzheimer's disease / dementia. In order to fully understand all-cause mortality by age group, it is important to track the specific causes of death. While life expectancy has dropped during the pandemic, it is important to illustrate that not all of these excess deaths were due to COVID-19 and associated respiratory conditions, but also due to other causes.

Vaccines were developed rapidly to reduce the mortality due to COVID-19. Countless studies, including meta-analyses and systematic reviews, have demonstrated a reduced mortality rate among vaccinated individuals [2,3]. However, the component studies differ dramatically in their statistical assumptions, inclusion/exclusion criteria, clinical definitions, duration of follow-up, and examination of covariates like age, comorbidities, number of vaccines and vaccine manufacturter. Thus, it is prudent to repeat these analyses on a larger scale.

While many studies have suggested that the vaccine has saved a dramatic number of lives, other reports contradict that view. Sucharit Bhakdi, MD and Arne Burkhardt, MD wrote a report[4] where they reported on the pathology of individuals who died post vaccination, suggesting the vaccine was the cause of death. Pathologist Dr. Ryan Cole also has linked an uptick in cancers with the vaccine [5]. The EU has stated that a warning about adverse events should accompany the vaccines [6]. A re-analysis of the original clinical trial data showed that adverse events were higher in the vaccinated group [7]. And while these reports do not always fall in peer-reviewed

---

[1] https://www.cdc.gov/nchs/nvss/vsrr/covid19/excess_deaths.htm?fbclid=IwAR035yFGOZe5IaK0Xd7pOzq1-GVeMVgK81m1JJTwO7ulhQNzCinuhl-dU0Q

[2] https://pubmed.ncbi.nlm.nih.gov/35113777/

[3] https://pubmed.ncbi.nlm.nih.gov/36091533/

[4] https://doctors4covidethics.org/wp-content/uploads/2021/12/end-covax.pdf

[5] https://podcasts.apple.com/us/podcast/dr-ryan-cole-alarming-cancer-trend-suggests-covid-19/id1471411980?i=1000550018801

[6] https://www.reuters.com/business/healthcare-pharmaceuticals/eu-regulator-says-novavax-covid-vaccine-should-carry-side-effect-warning-2022-08-03/

[7] https://www.sciencedirect.com/science/article/pii/S0264410X22010283

journals, they are worth noting. The best way to address these questions is with the analysis of a large national database.